H.R.1 Hospital Impact: Executive Brief

Nov 2, 2026 (v3, revised after independent review) · Hamza Saleh, MPH

Bottom line

In the Base scenario, H.R.1's Medicaid financing and eligibility provisions could reduce U.S. hospital margins by about **$56 billion a year** by 2032 (range $32–91 billion), or 3.5% of revenue. The industry's aggregate total margin would fall from 8.0% to about 4.7% (6.7% to 3.2% excluding investment income), and roughly 550–700 hospitals that are profitable today would show a loss. The cumulative 2026–2034 impact is about $340 billion (range $200–535 billion).

Three findings matter most for leadership:

1. **State-directed payment caps, not coverage loss, are the biggest exposure.** They are about two-thirds of the 2032 impact and keep growing until programs reach the new limits, around 2035 nationally. Coverage loss arrives first (2027) and is about a quarter of the 2032 impact. The provider-tax limit nets to only about $2.6 billion for hospitals overall, because hospitals also stop paying the tax, but it moves money away from high-Medicaid hospitals.
2. **Exposure is concentrated, not uniform.** Hospitals in Medicaid expansion states lose 4.0% of revenue versus 2.0% in non-expansion states. The top Medicaid-share quartile loses 8.0% and slips to a −2.5% aggregate margin; the lowest quartile loses 0.4%. Louisiana, Kentucky, New Mexico, Oregon and Arizona face 7–11% revenue hits.
3. **The window to act is 2027, not 2032.** Work requirements and six-month renewals start nationally in January 2027 and the SDP phase-down in January 2028. Hospitals that adjust payer strategy and cost base during 2027 keep the most options. Medicaid and CHIP enrollment is already down 6.7% since June 2025, mostly for reasons that predate H.R.1, and it is the first number to track.

What H.R.1 changes for hospitals

H.R.1 (P.L. 119-21, signed July 4, 2025) reaches hospital income statements through three separate channels. Each hits a different kind of hospital on a different schedule, so the model treats them separately rather than as one flat "Medicaid cut."

Channel	What changes	Who is hit hardest	Starts	Full effect	Base impact, 2032
State-directed payment (SDP) caps	Managed-care payments capped at 100% of Medicare (expansion states) or 110% (non-expansion); grandfathered amounts	Safety-net and academic hospitals in states with large SDP programs (CA, IL, KY, TX, NC, LA, AZ, MI)	Jan 2028	~2035 (CMS proposed rule)	$37.0B

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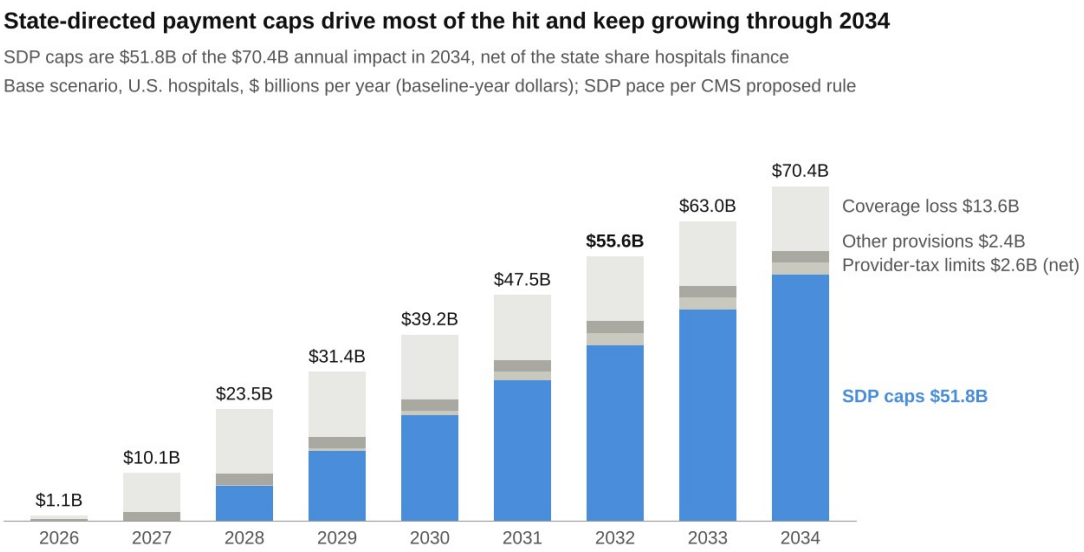
Channel	What changes	Who is hit hardest	Starts	Full effect	Base impact, 2032
	step down each year from 2028				
Provider-tax limits	Expansion-state threshold falls from 6% to 3.5%. Hospitals lose the payments the tax funded but stop paying the tax, so the net effect is redistributive	High-Medicaid hospitals in the 28 expansion states with hospital taxes above 3.5%; low-Medicaid hospitals can gain slightly	FY2028	FY2032	$2.6B (net)
Coverage loss	Work requirements (80 hours a month) and 6-month redeterminations for expansion adults; paid care becomes bad debt and charity	High-Medicaid hospitals in expansion states, especially rural and ED-heavy	2026 (NE, MT, AR, IA); all by Jan 2027	2028	$13.6B
Other eligibility provisions	Retroactive coverage cut to 1–2 months (Jan 2027); immigrant eligibility narrowed (Oct 2026); expansion-adult cost sharing (Oct 2028); MCO-tax uniformity (transition through 2028)	High-Medicaid hospitals; hospitals in MCO-tax states (CA, NY, IL, MA, MI, OH, WV)	Oct 2026	2029	$2.4B

The counterweight is the **$50 billion Rural Health Transformation Program** ($10 billion a year, FY2026–2030). The money goes to states for transformation projects: payments to providers are capped at 15% of each state's award, and FY2026 awards ranged from $147 million to $281 million per state. The model shows it beside the impact rather than netting it out. For rural leaders it is a source of restructuring capital, not operating support.

National findings

The impact builds in steps that follow the statute and CMS's 2026 proposed rules: coverage loss arrives in 2027, the provider-tax phase-down runs FY2028–2032, and grandfathered SDPs step down each year from January 2028 until they reach the new limits.

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H.R.1 Hospital Impact Model v3, Base scenario · 4,556 hospitals · CMS cost reports; KFF, CBO and CMS calibration

- **Margins:** the aggregate total margin falls from 8.0% at baseline to 4.7% in 2032 and 3.7% in 2034 (Base). Excluding investment income (cost report G-3 line 7), the margin falls from 6.7% to 3.2%. Hospitals had only just rebuilt margins: 3.9% in 2022, 7.3% in 2023, 8.1% in 2024 (total margin).
- **Hospitals in the red:** 1,293 hospitals already have negative margins (data-flagged rows excluded). That rises to about 1,860 in 2032 (Base), 1,630 (Low) or 2,180 (High). About 570 are pushed from profit to loss in Base (range 330–890); some 130 of those start with margins under 1%, so treat the count as indicative, not precise.
- **Scenario range:** the 2032 impact is $32 billion (Low), $56 billion (Base) or $91 billion (High). The spread comes mainly from how much capped SDP money states redirect to hospitals, how much of the SDP state share hospitals fund themselves, and how many adults lose coverage.
- **Enrollment is falling, for several reasons.** Medicaid and CHIP enrollment fell from 78.2 million (June 2025, sum of state reports; CMS's national figure was 77.7 million) to 72.9 million (July 2026), down 6.7%. Most of that predates H.R.1's coverage provisions; only Nebraska had started work requirements before July 2026. It is still the first number that turns into uncompensated care, so it is worth tracking monthly.
- **Calibration:** Coverage loss in Base (5.9 million adults, 30% of the 19.8 million expansion group) matches CBO's roughly 6 million from work requirements and redeterminations. The model's federal-equivalent SDP reduction for 2026–2034, before netting, is about $210 billion, above CBO's $149 billion 10-year estimate, partly because CMS's proposed rule phases payments down faster than the statute. The provider-tax channel nets to about $23 billion for hospitals over 2026–2035 before removing SDP overlap, against CMS's estimate of a $56.6 billion net loss for all provider types. The four smaller provisions in channel 4 are each calibrated to CBO's federal savings. Association estimates (CHA, AHA) measure gross federal funding cuts, not margin impact, so they are not direct comparisons.

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Who is most exposed

Exposure follows three fault lines: expansion status, the size of a state's SDP program, and how far its hospital tax sits above the new 3.5% limit. Nine of the ten most-exposed states are expansion states (Mississippi is exposed through its SDP program). Five have KFF state-level SDP estimates; the others' SDP amounts are allocated from KFF's state spending data.

State	2032 impact ($B)	% of hospital revenue	Margin: baseline → 2032	Hospital tax band (KFF)	Hospitals pushed to loss	Critical-tier hospitals	Medicaid enrollment change, Jun 2025–Jul 2026 (actual)
Louisiana	2.5	10.8%	10.8% → 0.0%	3.51–5.5%	48	28	−15.3%
Kentucky	2.7	10.6%	10.9% → 0.3%	3.51–5.5%	32	9	−7.6%
New Mexico	0.9	9.8%	9.5% → −0.4%	5.51–6.0%	6	7	−3.7%
Oregon	1.5	7.5%	0.3% → −7.7%	5.51–6.0%	13	16	−4.4%
Arizona	2.0	7.2%	10.9% → 4.1%	5.51–6.0%	21	7	−10.1%
Mississippi	0.8	7.1%	10.6% → 3.7%	Non-expansion (frozen)	27	32	−2.5%
Nevada	0.7	6.7%	12.8% → 6.5%	5.51–6.0%	9	6	−5.7%
Illinois	3.3	5.6%	6.5% → 0.9%	3.51–5.5%	27	43	−6.1%
Virginia	2.0	5.5%	15.3% → 10.4%	5.51–6.0%	13	10	−7.6%
North Carolina	2.8	5.4%	11.0% → 5.9%	3.51–5.5%	10	12	−2.7%

Base scenario, 2032. California has the largest dollar impact ($8.8 billion, 4.7% of revenue) and the most critical-tier hospitals (75). Oregon stands out because its thin baseline margin leaves no cushion.

By hospital segment (Base, 2032 impact as % of revenue):

- **Safety-net exposure.** Hospitals in the top quartile of Medicaid share (including supplemental payments) lose 8.0% of revenue and fall from a 5.8% to a −2.5% aggregate margin; the lowest

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quartile loses 0.4%. The top quartile accounts for 323 of the 571 hospitals pushed to loss. SDP losses are allocated by Medicaid revenue, so this gradient partly reflects the method; actual SDP preprints may concentrate it more or less.

- **Expansion vs non-expansion.** 4.0% vs 2.0%. Expansion-state hospitals fall from a 6.9% to a 3.0% margin; non-expansion states are exposed almost entirely through SDP caps, because their provider taxes are frozen, not cut.
- **Rural and critical access.** Rural hospitals (HRSA definition plus critical access) lose 3.7% of revenue against 3.5% for urban hospitals, and start from a thinner 6.9% margin, landing at 3.3% against 8.2% to 4.9% for urban. Critical access hospitals land at 3.8%.
- **Ownership.** Government hospitals take the largest relative hit (4.5%) from the thinnest base (6.0% margin falling to 1.6%), then for-profits (3.7%) and nonprofits (3.2%). Nonprofits account for 352 of the 571 hospitals pushed to loss.
- **Quality and finances move together.** One-star hospitals have a median Medicaid share of 18.1% (including supplemental payments), a median margin of 3.8%, a vulnerability index of 62 and a readmission ratio 3% above expected. Five-star hospitals sit at 8.3%, 8.2%, 41 and 3% below expected. This is a correlation, not a forecast of outcomes, but the hospitals with the least financial room are also the ones carrying quality penalties.

Risk tiers: 765 hospitals land in the Critical tier by 2032 (projected margin below −5% with weak liquidity), up from 541 at baseline. Another 1,134 are High risk. Facility-level cash is unreliable for system members, so treat tiers as a screen, not a credit view.

Strategic recommendations: an implementation playbook

The sequencing matters more than any single action. Coverage loss arrives first and is the cheapest to blunt; SDP and provider-tax losses arrive later but are larger and need structural answers that take two to three years to execute. For the average hospital, closing the 2032 gap means finding savings or new revenue worth about 3.5% of revenue. For a top-quartile Medicaid hospital, or any hospital in Louisiana, Kentucky, New Mexico, Oregon or Arizona, the figure is closer to 7–11%.

Phase	Action	Owner	Done by	Measure of success
1. Diagnose (now)	Run a hospital-specific H.R.1 stress test: replace this model's allocated SDP and provider-tax estimates with your state's actual SDP preprints and tax rates	CFO, Strategy	Q1 2027	Board-approved exposure range by channel and year
1. Diagnose (now)	Set up monthly tracking of Medicaid enrollment, churn and self-pay mix at the facility level, against the state trend	Revenue cycle	Q1 2027	Dashboard live; variance reported monthly
2. Protect coverage	Stand up enrollment and work-requirement support: financial	Revenue cycle, Patient	Jan 2027	% of Medicaid patients retained

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Phase	Action	Owner	Done by	Measure of success
(2027)	counselors, presumptive eligibility, help documenting exemptions (medical frailty, caregiving)	access		at redetermination; uninsured share of ED visits
2. Protect coverage (2027)	Re-tier the charity-care and self-pay policy for the newly uninsured; route eligible patients to Marketplace coverage	CFO, Patient financial services	Q2 2027	Bad debt as % of revenue held at baseline
3. Rebuild the cost base (2027–2029)	Launch a Lean / Six Sigma cost program sized to the projected 2030 gap: length of stay, supply chain, labor productivity, contract labor	COO, CNO	Start Q2 2027; 3-year run rate	Cost per adjusted discharge; % of 2030 gap closed
3. Rebuild the cost base (2027–2029)	Move care to lower-cost settings: observation, ambulatory surgery, care management for frequent ED users	CMO, Strategy	2028	Avoidable admissions; outpatient revenue share
4. Replace supplemental revenue (2028–2031)	Work with the state hospital association on financing that survives the caps: SDPs redesigned within Medicare-level limits, quality-linked payments, DSH and UPL optimization	CEO, Government affairs	Before Jan 2028 rating periods	Share of current SDP dollars retained
4. Replace supplemental revenue (2028–2031)	Renegotiate Medicaid managed-care and commercial contracts; pursue value-based arrangements that pay for the quality work already done	CFO, Managed care	2028–2029 contract cycles	Net revenue per adjusted discharge
5. Restructure (2029–2032)	Rationalize service lines with negative contribution margin; pursue regional partnerships, affiliations or Rural Emergency Hospital conversion where volumes no longer support inpatient care	CEO, Board	2030	Projected 2032 margin above 0%; days cash above 60
5. Restructure (2029–2032)	Rural systems: compete for Rural Health Transformation Program funds (telehealth, workforce,	Strategy, Rural health	Annual state RHTP	RHTP dollars secured per

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Phase	Action	Owner	Done by	Measure of success
	shared services) to pay for the restructuring	lead	cycles, FY2026–2030	facility

By segment:

- **Safety-net and academic hospitals in SDP-heavy states** (KY, LA, AZ, NC, IL, MI, CA): phase 4 is the biggest lever. Start state financing conversations in 2027; the annual phase-down is predictable, so build a glide path into the 2028–2032 budgets.
- **Rural and critical access hospitals:** the dollar amounts are small but the cushions are thinner. Pair the cost program with RHTP applications and early partnership talks.
- **Systems with mixed portfolios:** run the stress test facility by facility. Aggregate system margins hide the facilities that tip negative, and those are where service-line and capital decisions must be made first.

Method, limitations and validation

Scenario estimates, not forecasts. Every figure after the baseline is a projection. Most H.R.1 financing provisions take effect between 2027 and 2032 and are not yet visible in any CMS dataset.

- **Baseline:** 4,556 short-term acute and critical access hospitals, using each one's latest CMS Hospital Provider Cost Report (fiscal years ending Oct 2023–Oct 2025), joined on CCN to Care Compare, HAC, VBP and readmissions (HRRP) FY2026 results and Medicare inpatient data. Financials are as reported to CMS, not audited statements. Rural status uses HRSA's FORHP rural ZIP code file plus critical access designation.
- **Supplemental payments and margins:** Medicaid revenue adds cost report S-10 line 5 (supplemental and DSH payments not in line 2, net of taxes) from CMS's raw HCRIS files, and a margin excluding investment income (G-3 line 7) is shown beside the total margin. State federal shares come from MACPAC Exhibit 16 (FY2024).
- **Coverage channel:** each state's actual expansion-group enrollment (CMS, June 2025) × coverage-loss rate (Low 20% / Base 30% / High 35%; Base matches CBO) × the state's Medicaid spending per expansion adult (MACPAC Exhibit 23, FY2024) × a 41% hospital share (MACPAC Exhibit 19) × a net-loss factor (0.50 / 0.65 / 0.80), allocated by each hospital's share of state hospital Medicaid revenue.
- **SDP channel:** KFF's federal estimates of hospital SDP spending above the new limits for eight states; the rest of KFF's $60 billion national total is allocated in proportion to each state's actual SDP spending (KFF Appendix Table 1), so the ten jurisdictions without SDPs get none. Allocated within each state by Medicaid revenue including supplemental payments (cost report S-10 lines 2 and 5). Converted to all funds at each state's actual SDP federal share, then netted for the state share that hospitals fund through taxes and IGTs (Low 75% / Base 50% / High 25% of it), and reduced by what states redirect to hospitals (40% / 20% / 0%). The phase-down follows CMS's May 2026 proposed rule; the statutory reading is a switch in the model and lowers the 2032 Base to $52 billion.

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- **Provider-tax channel:** each expansion state's hospital-tax band from KFF's Appendix Table 1 (12 states at 5.51–6.0%, 16 at 3.51–5.5%, 13 with no hospital tax above 3.5%), using band midpoints applied to net patient revenue. Each hospital loses its share of the payments the tax funded (by Medicaid revenue including supplemental payments) but stops paying its share of the tax (by revenue), so low-Medicaid hospitals can come out slightly ahead. Tax dollars that fund SDPs are removed to avoid double counting (Low 60% / Base 40% / High 20%). Calibrated to CMS's July 2026 provider-tax impact analysis.
- **Other provisions channel:** retroactive-coverage limits, immigrant-eligibility limits, expansion-adult cost sharing and MCO-tax uniformity, each calibrated to CBO's federal savings ($4B, $6B, $7B and $35B over 10 years), converted to all funds and to hospital revenue with stated shares.
- **Vulnerability index:** weighted percentile ranks of Medicaid dependence (15%), projected H.R.1 hit (25%), three-year average margin (25%), liquidity (10%), uncompensated care (15%) and quality risk (10%: star rating, HAC penalty, VBP score and FY2026 excess readmission ratio). This is the analyst's method, not an established credit model.

Limitations. Hospital-level SDP exposure is allocated, not observed, for all but eight states; validate against your state's SDP preprints before any facility decision. Supplemental payments come from each hospital's latest cost report and are scaled to its baseline year. KFF reports hospital-tax rates only by band. Days cash is understated for system members. Channel 4 (retroactive coverage, immigrant eligibility, cost sharing, MCO-tax uniformity) is calibrated to CBO's national savings and allocated by Medicaid revenue, so its state detail is approximate. Still excluded: the emergency Medicaid match change (a state cost), the proposed extension of Medicare-based limits to all services (2029), ACA Marketplace changes and the end of enhanced premium tax credits, and any management response. For context: the Medicare PAYGO sequester was cancelled in November 2025 (P.L. 119-37), and Medicaid DSH cuts are delayed to FY2029 and reduced to $8 billion.

How to validate after implementation. As post-2027 cost reports arrive, compare margin and uncompensated-care changes in high- and low-exposure hospitals (difference-in-differences, controlling for state, size and ownership). That moves the analysis from projected to measured impact. Monthly Medicaid enrollment is the earliest indicator to watch.

Companion tools: an Excel scenario model (v3) (live Low/Base/High selector, state and hospital drill-downs), a Power BI build kit, and the interactive H.R.1 Hospital Impact Monitor dashboard.

Sources

- CMS: Hospital Provider Cost Report, Hospital General Information, HAC Reduction Program, Hospital Readmissions Reduction Program, Medicare Inpatient Hospitals by Provider, Medicaid & CHIP enrollment data
- RAND: State-Level Impacts of Key Medicaid Provisions in the OBBBA (2026)
- KFF: At Least 37 States Have Hospital SDPs That Could Be Reduced
- KFF: 5 Questions and Answers About Medicaid and Provider Taxes
- KFF: States' early work-requirement decisions
- Georgetown CCF: CBO coverage estimates of the reconciliation law
- HFMA: MedPAC hospital payment adequacy
- California Hospital Association: OBBBA impact analysis

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- Becker's: CMS Rural Health Transformation awards
- KFF: Spending on Medicaid State Directed Payments Before New Limits Take Effect (Appendix Table 1)
- SHVS: CMS releases state directed payment proposed rule (May 2026)
- HFMA: Medicaid provider-tax proposed rule and impact estimates (Jul 2026)
- HFMA: SDP proposed rule savings and overlap with provider taxes
- KFF: First-year Rural Health Transformation awards
- CMS: HCRIS Hospital 2552-10 cost report data files
- MACPAC: Exhibit 16, Medicaid spending by state and source of funds, FY2024
- KFF: Health provisions in the 2025 federal budget reconciliation law (CBO estimates by provision)
- CRS: Statutory PAYGO and Budget Reconciliation Legislation (Feb 2026)
- State inputs: KFF: FMAP for Medicaid, FY2027, CMS: Medicaid Enrollment – New Adult Group, MACPAC Exhibit 23 (FY2024), MACPAC Exhibit 19 (FY2022), HRSA: data files to identify rural areas

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